

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$24,741,024
Prenatal arm	\$11,855,074
Postnatal arm	\$12,885,950
Administrative load (8%)	\$1,979,282
Appropriation total (benefit + admin)	\$26,720,306
Sampling 90% CI	\$18,803,637 - \$30,678,411
Assumption band	\$14,844,812 - \$31,544,279

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Scenario	\$/birth	Cost	Appropriation	Births served	Medicaid %
Statutory - 3mo	\$3,000	\$24,741,024	\$26,720,306	8,766	91%
Statutory - 6mo	\$4,500	\$37,626,974	\$40,637,132	8,766	91%
Universal - 6mo	\$4,500	\$62,458,515	\$67,455,197	14,551	55%
Universal - 12mo	\$7,500	\$105,238,320	\$113,657,386	14,551	55%

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$38,657,850
Total cost (12-month design)	\$63,398,874

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

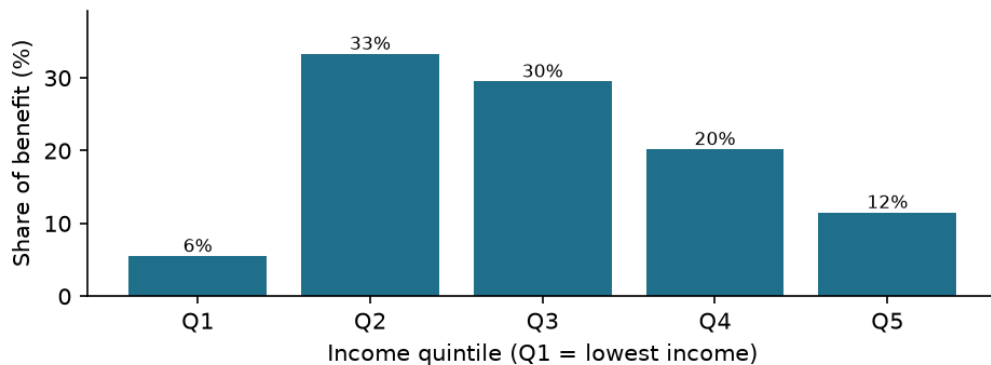
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$12,357,387
Prenatal / Postnatal	\$6,421,498 / \$5,935,889
Postnatal deferred to next FY	\$1,044,001
Year-1 as % of steady state	50%
Ramp sensitivity (6 / 12 / 18 mo)	\$18,512,815 / \$12,357,387 / \$8,238,258

Household reach

Births served / year (people affected)	8,766
via Medicaid clause	7,969
via income-only clause (300% FPL)	0
Eligible families (weighted)	7,340
via Medicaid clause	7,340
via income-only clause (300% FPL)	0
Expected recipients / year	16,494
Expected pregnancies / infants	7,903 / 8,591
Avg benefit per recipient	\$1,500

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	911	\$1,366,653	5.5%
Q2	\$57,028	110,263	5,499	\$8,248,070	33.3%
Q3	\$96,722	110,317	4,862	\$7,293,167	29.5%
Q4	\$154,809	110,231	3,329	\$4,993,148	20.2%
Q5	\$334,087	110,343	1,893	\$2,839,986	11.5%

Cost by county

County	Cost total	Prenatal	Postnatal	Births served	Recipients	Medicaid %
Hawaii	\$5,350,262	\$2,563,667	\$2,786,595	1,896	3,567	91%
Honolulu	\$16,341,942	\$7,830,514	\$8,511,428	5,790	10,895	91%
Kauai	\$1,766,336	\$846,369	\$919,967	626	1,178	91%
Maui	\$1,282,483	\$614,523	\$667,960	454	855	91%

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.